

For the use of a Registered Medical Practitioner or a Hospital or a Laboratory only.	
	
FINAINTAS 5 (Finasteride Tablets 5 mg)	
Name and strength of active ingredient Finasteride 5 mg	
Dosage form Film Coated Tablet	
Product Classification <u>Testosterone-5-alpha reductase inhibitors</u> (ATC Code: G04CB01)	
Product Description Blue, 7 mm, round, biconvex, film coated tablets marked 'F5' on one side and plain on other side.	
Pharmacodynamic properties Finasteride is a competitive inhibitor of human 5 α-reductase, an intracellular enzyme which metabolises testosterone into the more potent androgen, dihydrotestosterone (DHT). In benign prostatic hyperplasia (BPH), enlargement of the prostate gland is dependent upon the conversion of testosterone to DHT within the prostate. Finasteride Tablets 5 mg is highly effective in reducing circulating and intraprostatic DHT. Finasteride has no affinity for the androgen receptor.	
Pharmacokinetic properties After an oral dose of ¹⁴C-finasteride in man, 39% of the dose was excreted in the urine in the form of metabolites (virtually no unchanged drug was excreted in the urine), and 57% of total dose was excreted in the faeces. Two metabolites have been identified which possess only a small fraction of the Type II 5 α-reductase activity of finasteride. The oral bioavailability of finasteride is approximately 80%, relative to an intravenous reference dose, and is unaffected by food. Maximum plasma concentrations are reached approximately two hours after dosing and the absorption is complete within 6-8 hours. Protein binding is approximately 93%. Plasma clearance and the volume of distribution are approximately 165 ml/min and 76 l, respectively. In the elderly, the elimination rate of finasteride is somewhat decreased. Half-life is prolonged from a mean half-life of approximately six hours in men aged 18-60 years to eight hours in men aged more than 70 years. This is of no clinical significance and does not warrant a reduction in dosage. In patients with chronic renal impairment, whose creatinine clearance ranged from 9-55 ml/min, the disposition of a single dose of ¹⁴C-finasteride was not different from that in healthy volunteers. Protein binding also did not differ in patients with renal impairment. A portion of the metabolites which normally is excreted renally was excreted in the faeces. It therefore appears that faecal excretion increases commensurate to the decrease in urinary excretion of metabolites. Dosage adjustment in non-dialysed patients with renal impairment is not necessary. There are no data available in patients with hepatic insufficiency. Finasteride has been found to cross the blood-brain barrier. Small amounts of finasteride have been recovered in the seminal fluid of treated patients.	
Indication/Usage Finasteride Tablets 5 mg is indicated for the treatment and control of benign prostatic hyperplasia (BPH) and for the prevention of urologic events to; - Reduce the risk of acute urinary retention - Reduce the risk of surgery including transurethral resection of the prostate (TURP) and prostatectomy.	
Finasteride tablets 5 mg causes regression of the enlarged prostate, improves urinary flow and improves the symptoms associated with BPH. Patients with an enlarged prostate are the appropriate candidates for therapy with Finasteride 5 mg tablet. Finasteride tablet 5 mg administered in combination with the alpha-blocker doxazosin is indicated to reduce the risk of symptomatic progression of BPH.	
Recommended Dose The recommended adult dose is one 5 mg tablet daily, with or without food. Finasteride Tablets 5 mg can be administered alone or in combination with the alpha-blocker doxazosin (see 'Pharmacodynamic properties'). Although early improvement in symptoms may be seen, treatment for at least six months may be necessary to assess whether a beneficial response has been achieved. Thereafter, treatment should be continued long term. No dosage adjustment is required in the elderly or in patients with varying degrees of renal insufficiency (creatinine clearances as low as 9 ml/min). There are no data available in patients with hepatic insufficiency. Finasteride Tablets 5 mg is contra-indicated in children.	
Route of Administration 5 mg orally	
Contraindication Hypersensitivity to any component of this product; women who are or may potentially be pregnant; children.	
Warnings & Precaution Increased Risk of Breast Cancer Breast cancer has been reported in men taking Finasteride [dose, mg] during the post-marketing period. Physicians should instruct their patients to promptly report any changes in their breast tissue such as lumps, pain, gynaecomastia or nipple discharge. Increased Risk of High-Grade Prostate Cancer Men aged 55 and over with a normal digital rectal examination and PSA ≤3.0ng/ml at baseline taking Finasteride 5 mg/day in the 7 year Prostate Cancer Prevention Trial (PCPT) had an increased risk of Gleason Score 8-10 prostate Cancer (Finasteride 1.8% vs placebo 1.1%). Similar results were observed in a 4- year placebo-controlled clinical trial with another 5- alpha reductase inhibitor (dutasteride, AVODART) (1% dutasteride vs 0.5% placebo). 5- alpha reductase inhibitors may increase the risk of development of high-grade prostate cancer. Whether the effect of 5 – alpha reductase inhibitors to reduce prostate volume, or study –related factors, impacted the results of these studies has not been established. General Patients with large residual urine volume and/or severely diminished urinary flow should be carefully monitored for obstructive uropathy. Effects on prostate-specific antigen (PSA) and prostate cancer detection No clinical benefit has yet been demonstrated in patients with prostate cancer treated with Finasteride Tablets 5 mg. Digital rectal examination, as well as other evaluations for prostate cancer, should be carried out on patients with BPH prior to initiating therapy with Finasteride Tablets 5 mg and periodically thereafter. Generally, when PSA assays are performed a baseline PSA>10 ng/ml (Hybritech) prompts further evaluation and consideration of biopsy; for PSA levels between 4 and 10 ng/ml, further evaluation is advisable. There is considerable overlap in PSA levels among men	

RFIN02I1642-FINAINTAS 5(Malaysia)PIL

Size : 140 x 210 mm (Front)

Colour : Pantone Black

Date : 31/01/20, 15/11/21, 28/02/22, 01/03/22

with and without prostate cancer. Therefore, in men with BPH, PSA values within the normal reference range do not rule out prostate cancer regardless of treatment with Finasteride Tablets 5 mg. A baseline PSA <4 ng/ml does not exclude prostate cancer.

Finasteride Tablets 5 mg causes a decrease in serum PSA concentrations by approximately 50% in patients with BPH even in the presence of prostate cancer. This decrease in serum PSA levels in patients with BPH treated with Finasteride Tablets 5 mg should be considered when evaluating PSA data and does not rule out concomitant prostate cancer. This decrease is predictable over the entire range of PSA values, although it may vary in individual patients. In patients treated with Finasteride Tablets 5 mg for six months or more, PSA values should be doubled for comparison with normal ranges in untreated men. This adjustment preserves the sensitivity and specificity of the PSA assay and maintains its ability to detect prostate cancer.

Any sustained increase in PSA levels of patients treated with finasteride should be carefully evaluated, including consideration of non-compliance to therapy with Finasteride Tablets 5 mg.

Percent free PSA (free to total PSA ratio) is not significantly decreased by Finasteride Tablets 5 mg and remains constant even under the influence of Finasteride Tablets 5 mg. When percent free PSA is used as an aid in the detection of prostate cancer, no adjustment is necessary.

Excipients

Patients with rare hereditary problems of galactose intolerance, the Lapp lactase deficiency or glucose-galactose malabsorption should not take this medicine.

Interaction with other medicaments

No clinically important drug interactions have been identified. Finasteride Tablets 5 mg does not appear to significantly affect the cytochrome P450-linked drug metabolising enzyme system. Compounds which have been tested in man include propranolol, digoxin, glibenclamide, warfarin, theophylline, and antipyrine and no clinically meaningful interactions were found.

Other concomitant therapy: Although specific interaction studies were not performed in clinical studies, Finasteride Tablets 5 mg was used concomitantly with ACE inhibitors, alpha-blockers, beta-blockers, calcium channel blockers, cardiac nitrates, diuretics, H2 antagonists, HMG-CoA reductase inhibitors, non-steroidal anti-inflammatory drugs (NSAIDs) including aspirin and paracetamol, quinolones and benzodiazepines without evidence of clinically significant adverse interactions.

Pregnancy and Lactation

Pregnancy:

Finasteride Tablets 5 mg is contraindicated in women who are or may potentially be pregnant.

Because of the ability of Type II 5 α -reductase inhibitors to inhibit conversion of testosterone to dihydrotestosterone, these drugs, including finasteride, may cause abnormalities of the external genitalia of a male foetus when administered to a pregnant woman.

No effects were seen in female offspring exposed *in utero* to any dose of finasteride.

Exposure to finasteride-risk to male foetus

Women should not handle crushed or broken tablets of Finasteride Tablets 5 mg when they are or may potentially be pregnant because of the possibility of absorption of finasteride and the subsequent potential risk to a male foetus (see 'Pregnancy').

Finasteride Tablets 5 mg tablets are coated and will prevent contact with the active ingredient during normal handling, provided that the tablets have not been broken or crushed.

Small amounts of finasteride have been recovered from the semen in subjects receiving Finasteride Tablets 5 mg; 5 mg/day. It is not known whether a male foetus may be adversely affected if his mother is exposed to the semen of a patient being treated with finasteride. Therefore, when the patient's sexual partner is or may potentially be pregnant, the patient should either avoid exposure of his partner to semen (e.g. by use of a condom) or discontinue Finasteride Tablets 5 mg.

Lactation:

Finasteride Tablets 5 mg is not indicated for use in women. It is not known whether finasteride is excreted in human milk.

Side effects/Adverse Reactions

Finasteride Tablets 5 mg is well tolerated. There was no evidence of increased adverse experiences with increased duration of treatment with Finasteride Tablets 5 mg and the incidence of new drug-related sexual adverse experiences decreased with duration of treatment. Post marketing experience: Male breast cancer.

Signs & Symptoms of overdose and Treatment

No specific treatment of overdosage with Finasteride Tablets 5 mg is recommended. Patients have received single doses of Finasteride Tablets 5 mg up to 400 mg and multiple doses of Finasteride Tablets 5 mg up to 80 mg/day for up to three months without any adverse effects.

Storage Conditions

Do not store above 30°C. Store the tablets in original container / pack.

Shelf life

36 months

Pack size

Blister of 10 tablets. Each Carton contains 3 such strip. (3 x 10T)
Finasteride Tablets 5 mg PVC/PVdC-Alu blister pack of 10 Tablets.
Each Carton contains 3 such strip.
(3 x 10T)

Manufactured by:

INTAS PHARMACEUTICALS LTD

Matoda-382 210, Dist. : Ahmedabad. INDIA

Product Registration Holder:

Jetpharma Sdn Bhd,
No.13, Jalan Rajawali 2,
Bandar Puchong Jaya, 47100 Puchong,
Selangor, Malaysia

Date of Revision: Jun 2024.

RFIN021642

RFIN021642-FININTAS 5(Malaysia)PIL

Size : 140 x 210 mm (Back)

Colour : Pantone Black

Date : 31/01/20, 15/11/21, 28/02/22, 01/03/22